SYLTRA HEALTH — Medical Document

Where the platform stands in relation to care, for whom, and within what limits

Version 0.1, working draft. SYLTRA ONE. For medical advisors and care providers.

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0. A boundary to begin with

SYLTRA HEALTH is technology that helps people understand and follow their everyday health, and reach the people who care about them at the right time. It is a support for care between visits, not a replacement for it. It does not diagnose, does not label a condition, does not set a dose, does not predict a crisis, and does not stand in for a doctor or a pharmacist. Read everything here inside that boundary; it governs every line.

1. Where it stands, precisely

Most of a patient's life passes outside the clinic. A short visit every few months, and between them a whole life of small details no one sees. What SYLTRA HEALTH does is capture the context of those days, a person's movement and sleep, the environment of their home, and their adherence to medication, and organise it into a calm picture that helps them and their family understand, and helps the care team have a clearer conversation when the visit comes.

The value here is not a new number, but continuity. That a gentle thread of follow-up runs between visits, reassuring when things are well, and quietly, with a clear reason, drawing attention when something changes that is worth asking about.

2. Who benefits, and the medical value for each

Older adults. Many value their independence above almost anything, and their families live between a wish to be reassured and a fear of intruding. The platform notices changes in the usual pattern, such as movement missing at a time it usually happens, helping with reassurance without turning the home into a space of surveillance. With full honesty about the limit: this is a tool for support and follow-up, not an emergency service, it does not guarantee that help arrives, and contacting emergency services or a care provider remains the right action at a real moment of need [1] [4].

Chronic conditions. A chronic patient lives on measurements, appointments and a run of small decisions. Organising readings on a timeline, beside the context of activity and sleep, makes the trend clearer than a single number and makes a clinic visit more useful. We do not change their medication or dose; the medical decision stays between them and their care provider [8].

People of determination. For us, access is a right at the heart of the design, not a later addition. Someone living with a motor, hearing or visual disability, difficulty speaking, sensory sensitivity, or difficulty with memory, finds an environment that understands their way, presents information through more than one channel, and leaves the decision fully with them. The principle is greater independence, not greater surveillance [7].

Everyday wellness and sport. For someone who cares about fitness and recovery, linking body data to the home environment explains patterns of sleep and recovery and supports better habits, without any therapeutic claim.

3. How we read change responsibly

We build each person a personal baseline and compare them to themselves rather than to a general average, because what is normal for one may be notable for another. We issue no deviation alert before a learning period of two to three weeks, and we count a change as a deviation only when at least two independent signals agree at the same time. This caution is deliberate, because a frequent false alarm does not merely annoy; it loses trust and makes people ignore even what matters [6]. And when data is missing, we treat it as a technical fault we report to the user, not as a negative health signal.

4. Medication adherence, from a medical angle

Non-adherence has a documented effect on outcomes and cost [2][3]. Part of it is not a shortage of medicine but a matter of timing. The pack's run-out date is a calculated fact, yet no one acts on it in time. What we offer is a precise reminder before run-out, reaching the person who often cares for the patient, raising the chance of a timely refill. This is behavioural and operational value, not a pharmaceutical intervention. We do not decide when medication is dispensed and do not change a dose; the pharmacist dispenses against a valid prescription.

5. Medical safety and limits of use

We repeat it because it matters most. The platform does not diagnose or treat, does not predict a crisis, does not monitor medically around the clock, and does not call emergency services automatically. Any worrying symptom or sudden deterioration is not the platform's business but a reason to seek medical care at once. We design the copy so it never implies otherwise, and review it before every launch so that what we say stays within the wellness scope. And every alert we send has a reason we can explain to the user in one sentence.

6. The role of the doctor and care provider

We see our data as support for the care conversation, not a replacement for it. It helps the patient arrive at the visit with an ordered picture rather than scattered numbers, and helps the care provider follow up more closely between visits, as the patient permits. Critical decisions and escalation stay within a plan the user controls, not automated emergency decisions. And the human always stays in the loop.

7. What we watch, and why we chose it

We start with general, understandable signals, such as movement, presence, sleep and the daily readings the user permits, and home-environment signals such as temperature, humidity and air quality, which are linked to comfort and sleep [5]. We choose what serves understanding and follow-up, not merely what we can collect. And any new signal with health sensitivity is added with a clear justification and a separate consent.

8. Medical ethics considerations

We design around human dignity first. Autonomy means the person the data belongs to stays the decision-maker over who sees what and when. Non-maleficence means we are conservative in what we claim and explicit about limits, rather than overstating. Justice means access is a right for people of determination, not a privilege. And privacy is a condition, not a luxury, so we never share individual data with any entity that prices a service or coverage.

9. References

All references below are real and published. Verify their exact details and update them before any external publication or peer review.

1. World Health Organization. *World Report on Ageing and Health*. Geneva: WHO; 2015.
2. World Health Organization (Sabaté E, ed.). *Adherence to Long-Term Therapies: Evidence for Action*. Geneva: WHO; 2003.
3. Osterberg L, Blaschke T. Adherence to medication. *New England Journal of Medicine*. 2005;353(5):487–497.
4. World Health Organization. *Falls* (fact sheet). Geneva: WHO.
5. World Health Organization. *Housing and Health Guidelines*. Geneva: WHO; 2018.
6. The Joint Commission. *Sentinel Event Alert 50: Medical Device Alarm Safety in Hospitals*. 2013.
7. World Wide Web Consortium (W3C). *Web Content Accessibility Guidelines (WCAG) 2.1*. 2018.
8. World Health Organization. *Global Action Plan for the Prevention and Control of Noncommunicable Diseases 2013–2020*. Geneva: WHO; 2013.
9. IMDRF SaMD Working Group. *Software as a Medical Device (SaMD): Key Definitions*. 2013.
10. U.S. Food and Drug Administration. *Clinical Decision Support Software — Guidance for Industry and FDA Staff*. 2022.

10. Questions for the medical advisor

We raise them early and settle them together. Which signals suit each group best, and how to calibrate deviation thresholds so they stay meaningful and quiet. The most precise wording of the limits in the interface so it never implies a medical claim. The most useful shape of a follow-up report for the care provider without it turning into a diagnosis. And how to design the learning period for each case.

End of document. Read it within the no-medical-claim boundary at its start. We review anything touching medical use with a medical advisor before any launch.